طبع فى 19:16:33 11/10/2026

جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : دنيا حسن محمود عبدالفتاح

العنوان : 8 ش على عبد الله حسن الانور مصر القديمة القاهرة

المحافظة : القاهره **الحي / المدينة :** مصر القديمه

رقم البطاقة : 29203100103041 **رقم التليفون :** 01150500820 **تاريخ تسجيل الإستمارة :** 11-10-2026

العمر(تاريخ الميلاد) : 10-03-1992 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: دنيا حسن محمود عبدالفتاح
National ID: 29203100103041

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

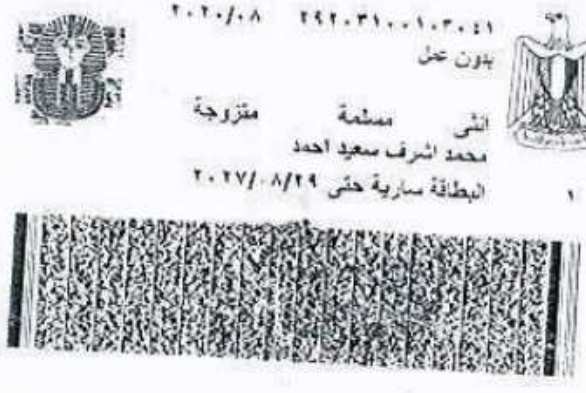
التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS

٢٢٠٢٦١٤١٢٢

* ضروس زولاوكس وأراضيكس



National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

08/26/2026

MDT FORM

Name:	Dona [illegible] Mahmoud .	MRN:	903
Age:	34 Y	ID:	29203100103041
Menopaus Status:	prem	Phone:*	01157147997
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender			
CO Morbidity:		Reason Of MDT: *	
		radiological revision of cts to be sure if there is pd or r	
Surgical History: *	none	ECOG Performance Status	
		○ 0 ● 1 ○ 2 ○ 3 ○ 4	

Clinical Data:

BX:

ER | PR | HER | KI6

History:

PREM RT BREAST CANCER MET TO LIVER ER//8 PR 8/8 HER2 FISH AMPLIFIED REC TAXOL CARBOPLATIN X18 WKS AND 5 HERCEPTIN CTS 2/12/2025 SD
pt roc up to 25 weeks taxol and 6 herceptin st zoladex el from 4/2026 and continued herceptin
pt is off tt for 2 months

cts on 13/8/2026 2 pulmonary nodules one nodule 10x8 mm other 1.4x.0.6 bilobar hft largest rt lobe seg 5 m 3.6x3 (mri 3/2026 in

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

PT OFF TIT FOR MORE THAN 3 MONTHS SO FOR PORTACATH
HERCEPTIN IV +XELODA
ECHO 7 3CYCLES
[illegible]

TNM

Right Side				Left S	
Q1	Q2	Q3	Q4	Q1	Q2

Clinical Staging:

Pathological Staging:

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Dunia Hassan Mahmoud Abdel Fattah Date : 17-08-2026 09:42
Branch : Dokki Medical No. : 441244
Ref.Physician Prof : Goustav Roussey Egypt

NUCLEAR MEDICINE BONE SCAN REPORT

CLINICAL INDICATION: 33-year-old female with right breast tumor. Currently on hormonal and targeted therapy. Presents with persistent bony aches. Referred for evaluation of skeletal metastases.

PROCEDURE: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of ^{99m}Tc-MDP.

FINDINGS:

- The axial and appendicular skeleton demonstrates homogeneous, symmetrical radiotracer distribution throughout the skull, spine, ribs, sternum, pelvis, and extremities.
- No focal areas of abnormal increased uptake or osteoblastic activity are identified.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

IMPRESSION:

- No scintigraphic evidence of osteoblastic skeletal metastases.
- Clinical follow up as per clinical pathway.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER

Call Center 19656

Patient's Name:	**Dunia Hassan Mahmoud Abdel Fattah**
Date:	**2026-08-13**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- **Known case of breast cancer with bilateral breast skin thickening & edema beneath it a bilateral breast masses seen with bilateral axillary enlarged lymph nodes with the largest in the right side measuring (1.9 x1.3mm) and the largest in the left side measuring (1.7x1.2cm) for Sonomamgraphy correlation.**
- **The right lung field showed 2 nodules, pulmonary nodule in the anterior segment of the right upper lung lobes measuring (10x 8mm) & the other one is right upper lobe posterior sub pleural nodule measuring (1.4x0.6cm) , yet clear left lung field with no detectable nodules.**
- **No consolidation or collapse.**
- **No hilar or mediastinal masses or lymph node enlargement.**
- **No pleural or pericardial collection.**
- **Normal plain CT appearance of the heart and great vessels.**

II- ABDOMEN AND PELVIS:

- **Mildly enlarged liver showing homogenous parenchymal yet rather with low attenuation. With noted hepatic bilobar multiple ill defined hypo enhanced focal lesions, the largest one is seen in the right lobe segment V measuring (3.6x3.0cm) …. Picture of Multiple metastatic lesions yet No dilated biliary radicles.**
- **Both kidneys are normal site, average size, and showing a smooth, regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.**
- **Normal CT appearance of the spleen, pancreas, suprarenal glands, aorta, and IVC.**
- **Normal CT appearance of pelvic organs with no pelvic masses or collections.**
- **IUD seen in place inside uterine cavity**
- **The urinary bladder shows no evidence of gross masses, diverticulae, or stones.**
- **No abdominal or pelvic lymphadenopathy.**
- **No ascites.**
- **Noted lower Lumbar vertebral mild spondylosis.**

CT Scan
[[DOCTYPE CT_SCAN]]

Patient's Name:	**Dunia Hassan Mahmoud Abdel Fattah**
Date:	**2026-08-13**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- **Noted L3 Lumbar vertebral body posterior small lytic lesion.**
- **Multiple lumbar vertebral body upper sclerotic lesions.**

Opinion:

- **Known case of breast cancer with bilateral Axillary Lymphadenopathy.**
- **Right Pulmonary metastatic nodules as detailed above.**
- **Fatty Hepatomegaly with multiple metastatic focal lesions.**
- **Noted L3 Lumbar vertebral body posterior small lytic lesion.**
- **Multiple lumbar vertebral body upper sclerotic lesions.**
- **Noted lower lumbar vertebral mild spondylosis.**

Best regards,

Dr. Ayman Shawky , FEBR

Kindly for Comparison

د/ أيمن شوقي

٢٠٢٦/٨/١٣

CT Scan
[[DOCTYPE CT_SCAN]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

04/20/2026

MDT FORM

Name:	Doha Kamal Mahmoud .	MRN:	903
Age:	34 Y	ID:	29203100103041
Menopausal Sta	prem	Phone:	01157147997
NRS SCORE:	pg 1	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: idc

ER: 7/8 PR: 8/8 HER2: 3+ KI67:

Sonomammography:

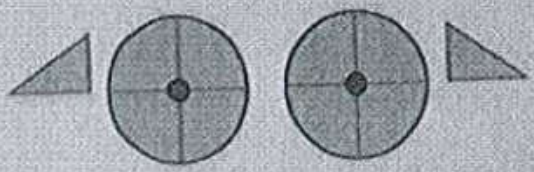
Mets Workup:

History:
PREM RT BREAST CANCER MET TO LIVER ER7/8 PR 8/8 HER2 SISH AMPLIFIED REC TAXOL CARBOPLATIN X18 WKS AND 5 HERCEPTIN CTS 2/12/2025 SD OF BILOBAR H/L LARGEST AT LEFT LOBE M 4.8X1 CM FOR DYNAMIC MRI BONY LESIONS FOR BS FOR pt rec 7 cycles herceptin and 27 weeks taxol mri abdomen 9/3/2026 left hfl seg 2 m 2.8 cm few smalls rt hfl largest seg 5 m 13 mm for mdt

Recommendations:

MDT Outcome & Plan
HERCEPITAN +ZOLADEX+AI

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name:	دينا حسن محمود عطاالله		
MRN:	124010220		
D.O.B:	10/03/1992	Location:	External Clinic
NIC No.:	29203100103041	Sex:	Female

Surgical Pathology-Diagnosis

Slide/Block revision Pathology

Specimen Id	2408190184		
Accession No	P-24-10473	**Collection Date**	19/08/2024 07:59 AM
Sample Type	TISSUE	**Received Date**	19/08/2024 07:59 AM
		Doctor	

CLINICAL HISTORY

Referred material of tru cut biopsy from Rt. breast UOQ mass.

GROSS DESCRIPTION

One referred Hx & E stained slide labeled by patient's name in Arabic. Four referred immunostained slides for ER, PR, HER2/neu and Ki67. One referred paraffin block.

MICROSCOPIC DESCRIPTION

- Histologic type : Invasive breast carcinoma of no special type.
  Histologic grade : Grade II (Intermediate).
- Ductal carcinoma in situ (DCIS) : Not identified
- Nuclear grade : Grade II (Intermediate)
- Necrosis : Not identified.
- Lymphovascular emboli : Not identified.
- Perineural invasion : Not identified.
- Special studies (including hormone receptor) :

IMMUNOHISTOLOGY

ER is positive, showing moderate positive reaction for 95% of tumor cells, score 7/8.
PR is positive, showing strong positive reaction for 95% of tumor cells, score 8/8.
Her2/neu is amplified by SISH(average her2 neu copy number is more than 6).
Ki67 proliferation index is about 20%.

DIAGNOSIS

Referred material of tru cut biopsy from Rt. breast UOQ mass: invasive duct carcinoma.

Category

Malignant primary site (single tumor)

Assisting pathologist

Dr.Nahla Gouda

Signed by

Dr. Bassant Ahmed, MD (Lecturer)

Pathology Report
[[DOCTYPE.PATHOLOGY_REPORT]]

Patient Name:	ديانا حسن محمود عبدالفتاح		
MRN:	124010220		
D.O.B:	10/03/1992		
IC No.:	29203100103041	Location:	External Clinic
		Sex:	Female

Surgical Pathology-Diagnosis

Slide/Block revision Pathology

Specimen Id	2408190184		
Accession No	P-24-10473	**Collection Date**	19/08/2024 07:59 AM
Sample Type	TISSUE	**Received Date**	19/08/2024 07:59 AM
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Category

Malignant primary site (single tumor)

Assisting pathologist

Dr.Nahla Gouda

Signed by

Dr. Bassant Ahmed, MD (Lecturer)

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

د . هاني أحمد شامي
دكتوراه الأشعة التشخيصية
أستاذ بكلية طب القصر العيني

مركز الطاهرة للأشعة

TAHRA RADIOLOGY CENTER EST. 1991

صلاح علي الشيخ
ه الأشعة التشخيصية
كلية طب القصر العيني

Name: Mrs. Donia Hassan Mahmoud Abdel-Fattah

Date: 23/7/2024 ID:2211010232 Acc.:5013569

Referring Prof. Dr.:Anwar Al-Shenawy.

Ultrasound guided Tru Cut biopsy

- After complete aseptic condition, Multiple cores were obtain from a rig breast UOQ malignant mass lesion (BIRADS 5) histopathological correlatic and PR ,ER and HER2 .

Truly yours;
Dr. Yomna Tarek, MSc

Spelling checked By Dr YT
Typed A R 23/07/24

BI-RADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BI-RADS 0: Incomplete - need additional imaging evaluation or comparison with previous films
- BI-RADS 1: Normal examination - nothing to comment on - Routine mammography screening is recommended
- BI-RADS 2: Benign findings - routine mammography screening is recommended.
- BI-RADS 3: Probably benign <2% malignancy - short interval follow-up is suggested.
- BI-RADS 4: Suspicious >2 but <95% - Biopsy should be considered.
  - 4A: low suspicion of malignancy: 2-20%.
  - 4B: Mod suspicion of malignancy: 10-50%.
  - 4C: High suspicion of malignancy: 50-90%.
- BI-RADS 5: Highly suspicious of malignancy >95% - appropriate action should be taken
- BI-RADS 6: Known - biopsy proven malignancy. Surgical excision when clinically appropriate.

American college of radiology (ACR) scoring of breast density:

- ACR a: almost entirely fatty.
- ACR b: scattered areas of fibro-glandular density.
- ACR c: heterogenously dense.
- ACR d: extremely dense.

HOTLINE 19173

G-CERT

Breast Biopsy

[[DOCTYPE:BREAST_BIOPSY]]

د . هاني أحمد سامي
دكتوراه الأشعة التشخيصية
أستاذ بكلية طب القصر العيني

سعاد علي الشيخ
دكتوراه الأشعة التشخيصية
أستاذ بكلية طب القصر العيني

Name: Mrs. Donia Hassan Mahmoud Abdel-Fattah
Date: 27/5/2024 ID:2211010232 Acc.:4999059
Referring Prof. Dr.:Haytham Fekery Osman.

Ultrasound examination of both breasts, revealed:

Indication: right palpable lump.

- Right breast shows an ill-defined diffuse hypoechoic pathological altered echogenicity mounting to be a diffuse ill-defined solid mass lesion seen at the outer quadrants, extending from 11-12 o'clock to 8-9 o'clock, mainly at 9-10 o'clock from Zone C posteriorly to retro-areolar area anteriorly causing nipple retraction (corresponding to the main palpable lump), showing multiple internal cystic changes and internal echogenic foci? Calcific foci. The lesion is away from the skin by (3.4 mm). Mild to moderated diffuse skin thickening up to 3.2 mm with mild interstitial edema.
- No left definite solid or cystic lesions.
- No left dilated retro-areolar ducts.
- Normal areola / nipple complex on left side.
- Normal skin thickness and contour on the left side
- Right enlarged axillary lymph nodes at level I (at least 4) with thick cortex up to 6 mm, some with eccentric hilum and others with preserved hilum?? Indeterminate.
- Left non-specific axillary lymph nodes.

Opinion:

- Right outer quadrants malignant pathological process as described (BIRADS 5)

Recommendation:

- Ultrasound guided core biopsy.
- Dynamic MRI breast is highly recommended.

Truly yours
Dr. Samah Sayed, Msc

Spelling checked By AR

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- ACR_c: heterogenously dense.
- ACR_d: extremely dense

HOTLINE 19173
G-CERT
Ultrasound
[[DOCTYPE:ULTRASOUND]]

EGY SCAN

Name: Donia Hassan Mhmod*	ID: 468277
Date : 08/06/2024	Ref. Doctor: Haitham Fekry Othman

18 F-FDG PET/CT examination

Clinical Indication:

- *Patient with right breast malignancy for assessment.*

Examination protocol:

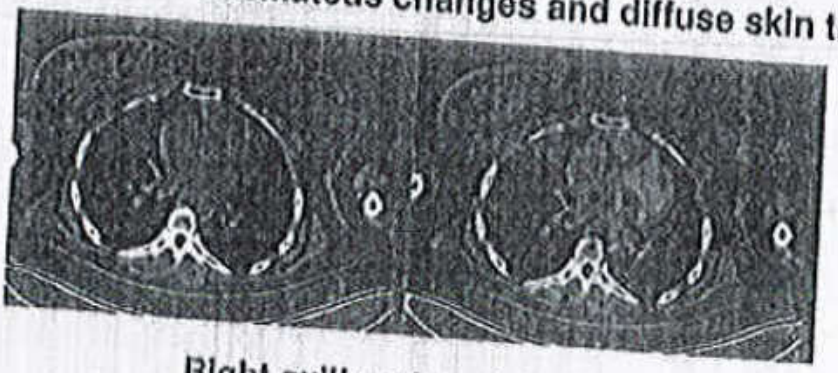
- A Whole body PET/CT study was obtained from skull base to the mid thigh using a low dose non contrast CT for attenuation correction
- Diagnostic non-contrast multislice CT examination of the same region.
- Average hepatic uptake~1.4.

Findings:

- Both breast and axilla:

  Right breast:

  - FDG avid right breast UOQ ill-defined soft tissue mass lesion is seen measuring 1.2x0.9 cm in axial dimensions with SUVmax~ 3.31. Associated diffuse right breast heterogenous CT density showing increased FDG uptake with mammary edematous changes and diffuse skin thickening.





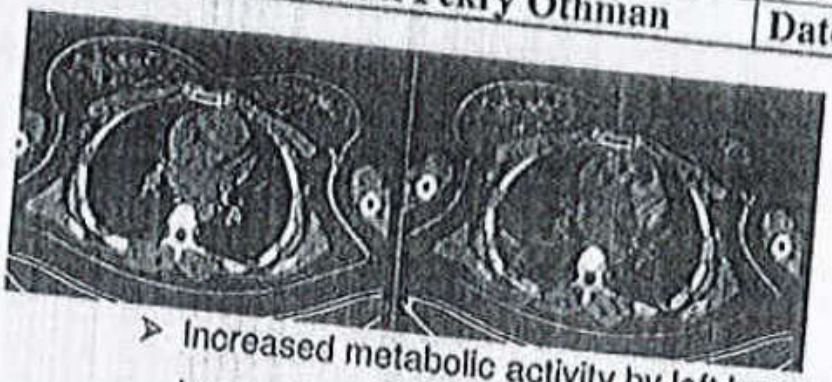
  Right axillary lymph nodes:

  - FDG avid few enlarged right axillary (level I, II & III) lymph nodes; the largest seen at level I measuring 1.6x1.1 cm in axial dimensions with SUVmax~2.1.

PET Scan
[[DOCTYPE:PET_SCAN]]

EGY SCAN

Name: Donia Hassan Mhmod*	ID: 468277
Ref. Doctor: Haitham Fekry Othman	Date : 08/06/2024





- Increased metabolic activity by left breast parenchyma with no left breast FDG avid lesion or pathologically enlarged axillary lymph nodes.

- Head and neck region:
  - Low grade metabolic activity by prominent right supraclavicular lymph nodes measuring 1.7x1 cm with SUVmax~1.0.
  - Low grade metabolic activity by prominent bilateral upper and lower deep cervical lymph nodes are also noted.
  - Heterogenous CT texture of the thyroid gland, no focal FDG avid lesions.
  - Normal FDG uptake within the salivary gland.
  - No focal brain FDG avid lesion.

- Lungs & Mediastinum:

  Lungs & Pleura:
  - No FDG avid pulmonary nodules.
  - Middle lobe atelectatic band.
  - No FDG avid pathological pleural thickening.
  - No pleural or pericardial effusion.

  Lymph nodes:
  - No FDG avid mediastinal or hilar lymph nodes.

Name: Donia Hassan Mhmod*	ID: 468277
Ref. Doctor: Haitham Fekry Othman	Date : 08/06/2024

- No gross cardiac abnormality. Normal FDG uptake by myocardium.

- **Abdomen and pelvis:**
  - Mildly enlarged liver with no FDG avid lesion.
  - Normal non-contrast CT appearance of the spleen, pancreas, kidneys and adrenal glands with no FDG avid lesions.
  - Increased FDG uptake by the endometrium, likely physiologic.
  - Left adnexal cyst is seen measuring 4.5x3.4 cm in axial dimensions, likely functional.
  - No FDG avid abdominopelvic lymph nodes.
  - No ascites.



- **Musculoskeletal system:**
  - No FDG avid osseous lesions.
  - Diffuse increased FDG uptake by the axial skeleton, likely element of bone marrow hyperplasia. For clinical and lab correlation.
  - No CT evidence of vertebral collapse or extra-osseous soft tissue component.

EGY SCAN

Name: Donia Hassan Mhmod*	ID: 468277
Ref. Doctor: Haitham Fekry Othman	Date : 08/06/2024

OPINION:

- Metabolically active right breast locally advanced malignant process with ipsilateral metabolically active metastatic axillary lymph nodes as descried.
- Otherwise no FDG avid distant metastatic lesions.

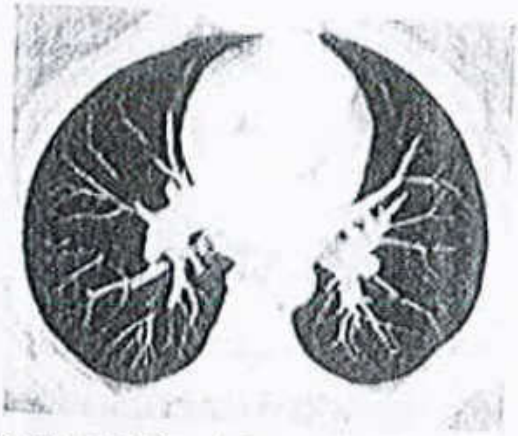
Regards,
Dr. Mahmoud Ghazy, MD

Name: Donia Hassan Mahmoud	ID: 562815
Ref. Doctor: Almahd Alkomi Llaoram	Date : 28/10/2024

Multislice CT scan of the Chest , abdomen and pelvis after oral and IV contrast administration with multiplannar reformatted images revealed:

Reported case of cancer breast revealed:

I . CHEST



- Bilateral few scattered subcintimetric pulmonary nodules reaching 3mm at the left lower lobe. No evidence of sizeable masses.
- No focal pulmonary parenchymatous consolidative patches, bronchiectatic changes or cavitary lesions seen.
- No CT evidence of pathological hilar or mediastinal lymph node enlargement.
- Enlarged right axillary lymphnodes; the largest reaching 1.2cm in short axis. Few subcintimetric left axillary nodes.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Right breast heterogeneous enhancing lesion with associated skin thickening.

OPINION:

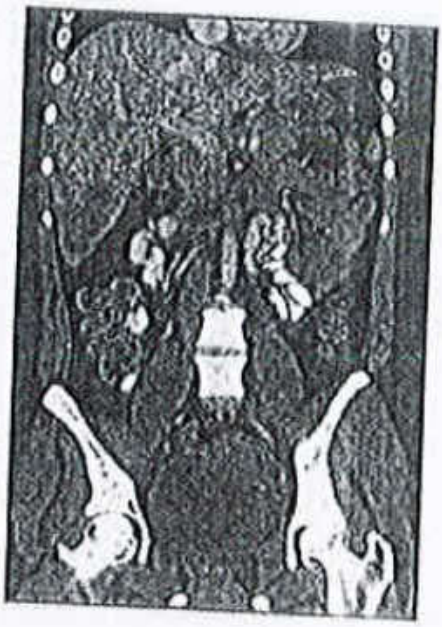
- **Bilateral few scattered subcintimetric pulmonary nodules; for follow up.**
- **Right breast heterogenous enhancing lesion with skin thickening and associated enlarged axillary lymphnodes; for sonomammographic correlation.**

CT Scan
[[DOCTYPE CT_SCAN]]

Name: Donla Hassan Mahmoud	ID: 562815
Ref. Doctor: Almahd Alkoml Llaoram	Date : 28/10/2024

II. ABDOMEN AND PELVIS

- Enlarged liver showing bilobar multiple variable sized hypoenhancing focal lesions in different study phases; the largest seen at segment V measuring about 3.5x3.7cm in maximum axial diameter.
- No intra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, adrenals, both kidneys, aorta, IVC and pelvic organs.
- Subcintimetric para-aortic, aortocaval and bilateral common iliac lymphnodes; the largest at the former reaching 8mm in short axis.
- No ascites.
- Average capacity of the urinary bladder showing smooth outline. No stones, masses or diverticualr outpouchings.



OPINION:

- **Hepatic multiple hypoenhnacing focal lesions as described; likely metastatic.**

Dr. Sarah Mohamed, MD

Dr. Sameh Saed , MD *Dr. Maged Mostafa, MD, FRCR*

CT Scan
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Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الاشعة

Patient Name: Donia Hassan Mahmoud Abd Al Fattah	Date : 02/12/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 33 Year(s)
Medical ID: 170/2025	Pat. ID: 133208

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

COMPARED TO LAST STUDY REPORT CURRENT STUDY REVEALS

1. **Chest:**
   - STAIONARY course as regard the right axillary lymph nodes largest 29 x17mm , notable left axillary small lymph nodes likely reactive
   - Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
   - No hilar or mediastinal lymph nodes.
   - No evidence of pleural effusion, pleural thickening or masses seen.
   - Normal cardiac size. No pericardial effusion detected.
   - Normal CT appearance of the thoracic aorta and the great vessels.
   - Normal appearance of the dorsal vertebrae.
2. **Abdomen and pelvis:**
   - The liver is average in size with multiple bilobar linear hypodensities largest seen at left lobe measuring 4.8 x 1 cm . lesions shows minimal pportal phase enhancement and delayed phase washout multiple notable hepatic capsualar retraction noted
   - Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
   - Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
   - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
   - Normal appearance of the other pelvic organs , IUD is seen in place
   - No abdominal or pelvic collections.
   - Multilevel lumber vertebral osteolytic lesions with sclerotic margins seen.

OPINION:

- ***Chest study showing genralized stationary course compared to last study.***
- ***Still seen with stationary course of multiple hepatic indeterminant linear hypodensity with capsular reteactions of abnormal enhancement for further Dynamic MR abdomen with DWI.***

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- ***Multiple lumber lytic lesions with sclerotic margins for correlation with bone scan***

Dr Abubakr Hejazi MSc FEBR

Signature Dr . Nermien Ramadan

16/12/2025 04:49 م

CT Scan
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